

LONGITUDINAL HEARING-CARE RELATIONSHIP INTELLIGENCE

A co-creation conversation for lifetime continuity, better outcomes and differentiated care

Proposed by Anupam Kumar • 12-month co-creation horizon



The clinic is running well. This is about becoming harder to imitate.

Operational systems record visits and transactions. The opportunity is to preserve continuity and meaning across a lifetime.



What works today

Appointments
Billing and invoices
Inventory and repairs
Clinical expertise
Trusted patient relationships



What remains fragmented

Patient goals over time
Caregiver roles and consent
Life between appointments
Cross-provider hand-offs
Rehabilitation continuity



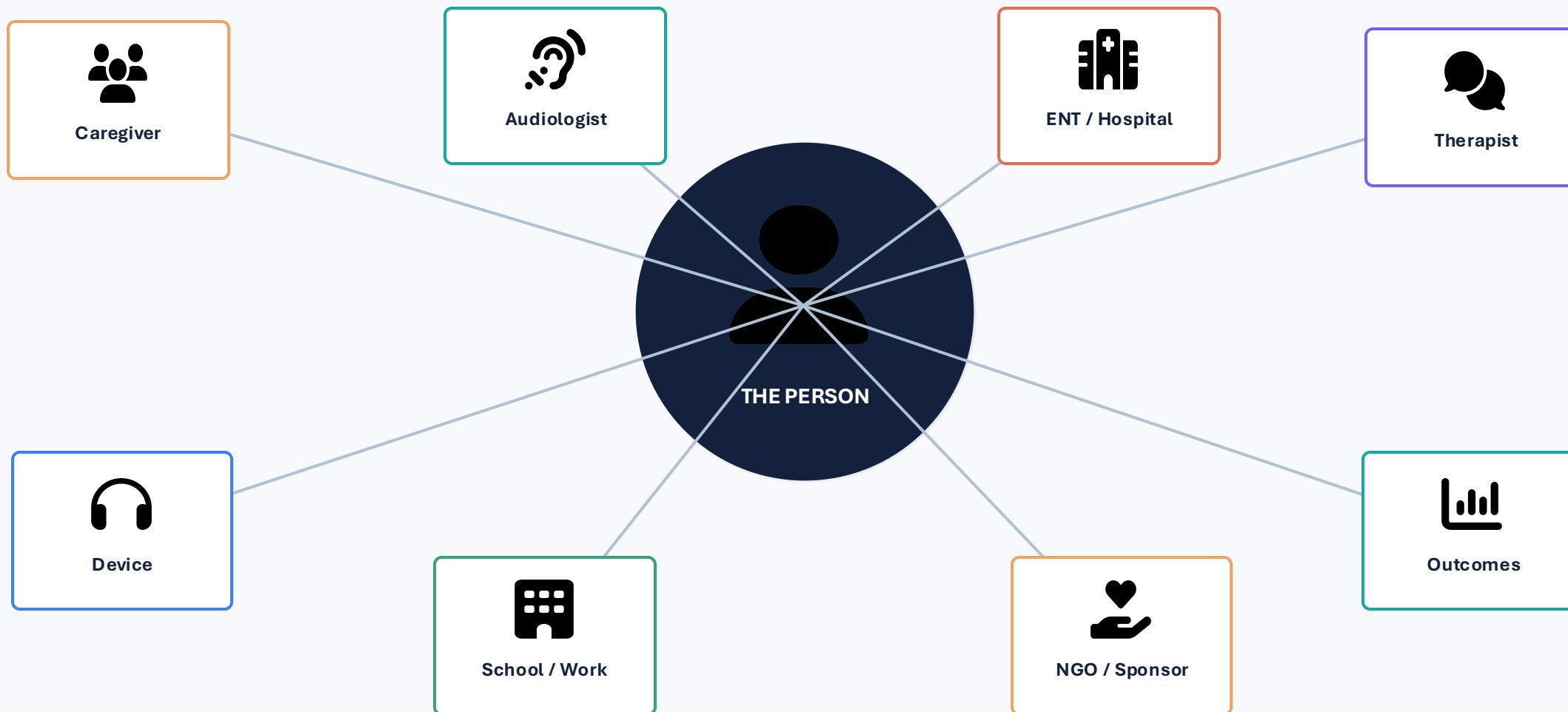
The additional muscle

One hearing journey
Owned follow-up signals
Device and downtime context
Outcome patterns
Differentiated lifetime care

The aim is not to fix failure. It is to compound excellence.

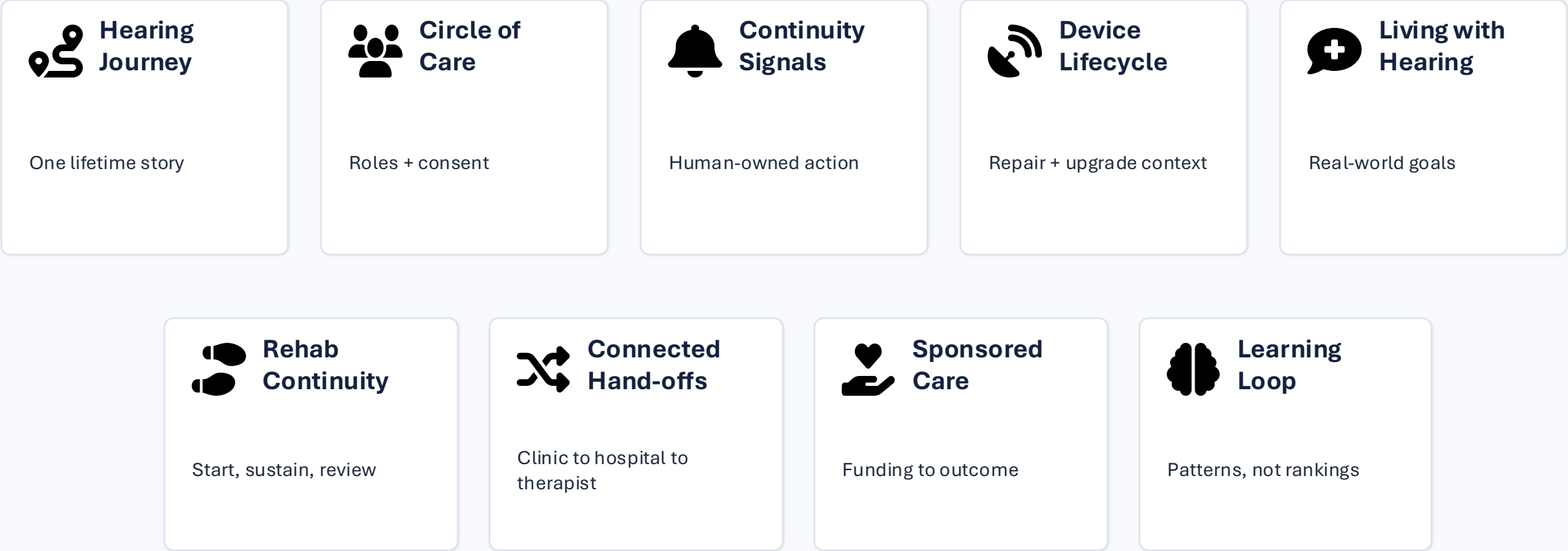
The missing view: the whole hearing-care relationship

A person is not an appointment, an invoice, an audiogram or a serial number.



What the intelligence layer would do

It works beside Practo, ERPNext, Audinexa, Noah and manufacturer systems. It does not replace them.



Humans decide. The system connects, remembers and surfaces.

How this helps patients and caregivers

The value must be visible outside the clinic, where hearing is actually lived.



The clinic remembers

Goals, preferences, accessibility needs and unresolved concerns travel with the person.



Caregivers become explicit

Chosen supporters have clear roles and boundaries instead of being invisible or overreaching.



Problems surface earlier

Device downtime, missed rehabilitation and unresolved adaptation become visible to a human owner.



Progress means daily life

Phone calls, school, work, family conversation, fatigue and confidence matter alongside test results.

How this helps the clinic team

The system should remove coordination work from audiologists, not add another documentation burden.



Reception / coordinator

Own routine follow-up, consent, hand-offs and unresolved queues.



Audiologist

See a concise longitudinal summary when clinical judgement is required.



Owner

See continuity, outcomes, referral quality and patient-base health.

DESIGN TEST

If the system requires audiologists to duplicate routine data, chase reminders or manage the CRM, the product has failed its own promise.

The intelligence layer succeeds only when it delivers a better clinical conversation with less administrative effort.

Quality insights the clinic can act on

Not more dashboards. Better questions that lead to owned action.

 Who has no next step?	Continuity	 Where do trials stall?	Journey
 Who is struggling after fitting or activation?	Adaptation	 Which CI recipients are dropping from follow-up?	CI continuity
 Where was rehab recommended but not sustained?	Rehabilitation	 Who is without a working device or loaner?	Device downtime
 Which referrers close the care loop?	Relationships	 Which outcomes improve over time?	Learning

Revenue can improve, but through better care

The commercial value is a consequence of continuity, not a sales engine.



Retention

Fewer patients disappear after trial, fitting, activation or repair.



Family loyalty

Caregivers experience ongoing support rather than a transaction.



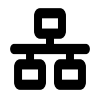
Legitimate upgrade reviews

Device age, repair burden and changing needs prompt a clinician-led review.



Outcome visibility

The clinic can demonstrate the quality of its service model.



Referral confidence

ENTs and hospitals receive appropriate closure when the patient consents.



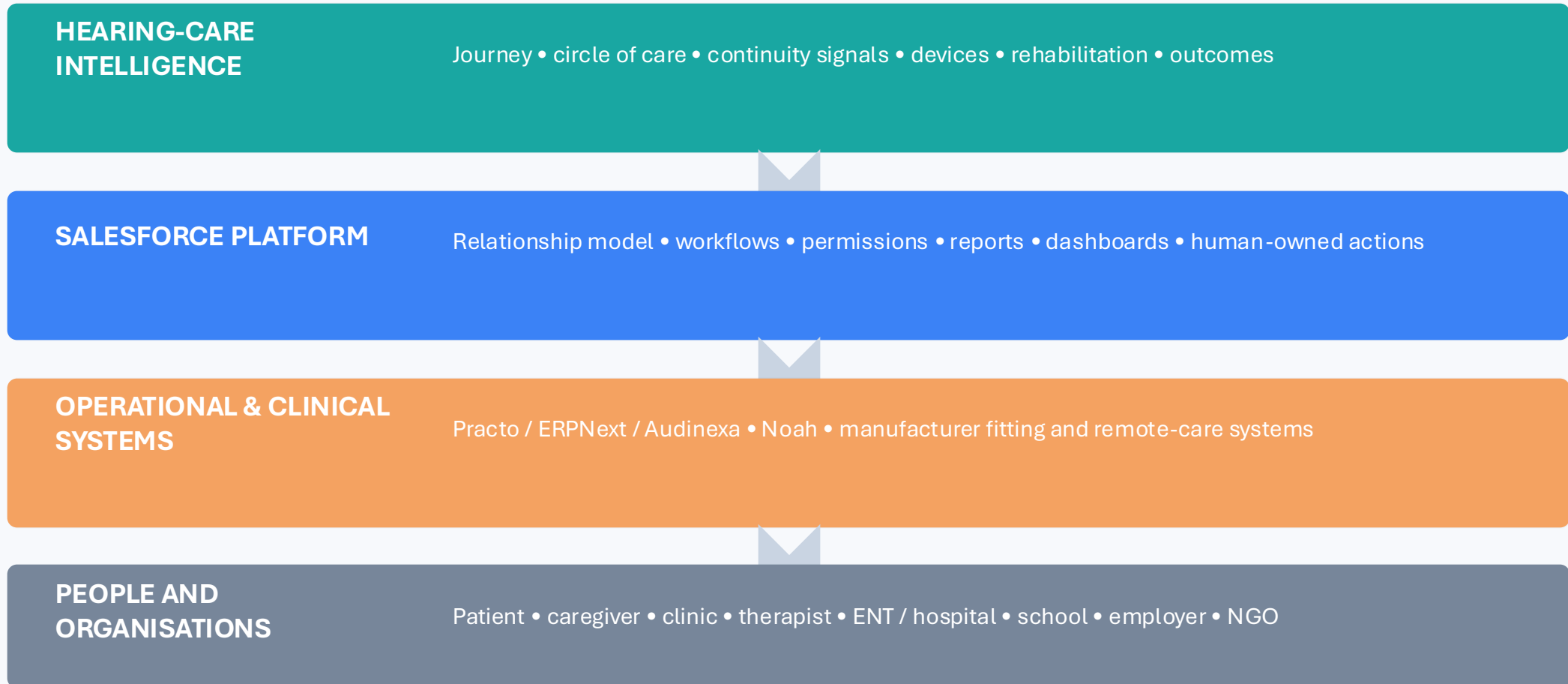
Sponsored care

The NGO can connect funding, device access, continuity and outcome.

Guardrail: never turn clinical care into an upgrade propensity score.

Where Salesforce fits

Salesforce is the co-creation platform. The product vision remains hearing-care intelligence.



Platform choice enables the experiment. It is not the proposition.

The co-creation question

Is this a capability the clinic wants to help define and become known for?

Can we make lifetime continuity visible without increasing clinical burden?



Patients & families

Would they feel more supported?



Audiologists

Would the view improve the clinical conversation?



Clinic owner

Would the insight strengthen differentiation?



NGO & partners

Would the evidence improve accountable access?

PROPOSED NEXT CONVERSATION

Refine the product vision with the clinic's audiologists, therapists, coordinator, patients, caregivers and NGO representatives before any product development begins.